THE

LOOKING GLASS

Lash & Brow Tint & Wax

Course Manual



Where Beauty Meets Alchemy

Course Details

Aims

The course aims to ensure you; the student understands the basics of health and safety and anatomy and physiology of the treatment. This manual covers the treatment background, benefits, consultation and contra-indications, contra-actions, aftercare and equipment and products required to perform the treatment. The practical techniques will be covered on the practical session to ensure competency in the procedure.

Objectives

At the end of the course, you will be able to perform a treatment in a professional, safe and hygienic manner in a commercially acceptable time, along with experience in carrying out a thorough consultation with the knowledge of the background, benefits, consultation, contra-indications, contra-actions, aftercare, equipment and the products needed.

Accreditation

This course is accredited by:

CPD Accreditation
https://gb.cpdstandards.com/

Insurance

Students will be able to gain insurance from the following provider(s) listed below, upon successful completion of your training:

Insurer name: Radius Insurance
Website: www.radiusinsurance.co.uk

Insurer name: salon insurance 4 U
Website: www.saloninsurance4u.co.uk

Medical Disclaimer

It is advised that you take medical advice if you or any of your clients have a health problem. Any qualification from The Looking Glass Beauty School will not qualify you to advise on or diagnose any medical condition.

Contact Details

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Health & Safety

You will need to maintain a high standard of hygiene as well as health and safety, not only for yourself but also for your employees, clients and any visitors to your business.

It is a legal requirement for employees to display an approved health and safety poster or to provide employees with an equivalent leaflet or information.

All businesses are required by law to comply with the following acts, which are monitored and managed by The Health & Safety Executive (HSE). You should also get copies of the following regulations from your local council or off the HSE website.

Health and Safety at Work Act 1974

This protects your rights as an employer or employee. The law states that the employer must provide a safe working environment, provide health and safety training for staff, produce a written policy of the company's health and safety policy and ensure that anyone on their premises is not exposed to any health or safety risks.

Trade Descriptions Act (1968 and 1972)

These Acts prohibit the use of false descriptions of goods or services. The information must always be accurate, false comparisons must not be made, and misleading price comparisons must not be made. A product may not be described as being of a 'reduced' price if it has not been available at a higher price for a minimum of 28 days.

General Data Protection Regulation GDPR

If you are collecting and storing personal data as a therapist, then you will need to comply with GDPR. You will need to decide which of the six lawful bases on which you will collect and store personal data and inform your clients of how and why you will retain their data and for how long. The Independent Commissioners Office will provide you with all relevant information.

Sale and Supply of Goods Act 1994

This states that goods must be as described and of satisfactory quality. They should be fit for purpose and safe for use. It is the responsibility of the retailer to correct a problem where the goods are not as described.

COSHH Regulations and Risk Assessment (Control of Substances Hazardous to Health)

COSHH regulations cover the essential requirements for controlling exposure to hazardous substances, and for protecting people who may be affected by them. You should carry out a COSHH assessment to identify all chemicals, products or other substances which could cause harm.

A substance is considered to be hazardous if it can cause harm to the body. It poses a risk if it is inhaled, ingested, in contact with the skin, absorbed through the skin, injected into the body or introduced to the body through cuts.

Always check the ingredients and instructions of all products to see what they contain and ensure they are correctly stored. If the product could cause harm, it

should be listed on your COSHH assessment, together with what the risk is and who is at risk from it.
Next, decide on the degree of risk and who to minimise that risk. If you can, try to replace high-risk products with lower risk ones. Never leave chemicals identified as hazardous in areas accessible to the general public. Do not forget, COSHH substances include both those used for treatments and cleaning.

Local Government (Miscellaneous Provisions) Act 1982
A special treatment licence will be required if you carry out any form of massage, electrolysis or ear piercing and tattooing as they may produce blood and body tissue fluid. Each borough council in the UK has different requirements, so you should contact them to see whether they require you to hold a licence for the treatments you offer.

The Management of Health and Safety at Work Regulations 1999
Employers should make formal arrangements for maintaining and improving safe working conditions and practices. This includes competency training and risk assessments.

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The Manual Handling Operations Regulations 1992
This is relevant wherever manual lifting occurs to prevent skeletal and muscular disorders. The employer should undertake a risk assessment for all activities involving manual lifting.

The Personal Protective Equipment at Work Regulations 2002
This requires employers to identify activities which require special protective clothing, which must then be made available.

The Health and Safety (Display Screen Equipment) Regulations 1992
This covers the use of display screens and computer screens. This specifies the acceptable levels of radiation emissions from the screen, as well as identifying the correct posture and the number of rest periods.

The Electricity at Work Regulations 1992
Electrical items are potentially hazardous and should be used and maintained properly. You should always ensure that you are fully trained on a piece of equipment before operating it.
All electrical equipment should be regularly PAT tested to ensure it is safe to use. If any equipment is deemed to be faulty or unsafe, you should stop using it immediately and report the problem. Make sure the equipment is clearly marked as faulty until the problem has been corrected to avoid it being used by other members of staff.

Health and Safety (First Aid) Regulations 1981
Whatever the size of your business, you should always make sure you have a First Aid kit on-site, as well as an eyewash bottle. You should ensure this is fully stocked at all times. You should have at least one 'Appointed Person' on hand to take charge in an

emergency who holds an HSE-approved basic first aid qualification. You can contact the HSE on 0845 345 0055 for a list of suitable training providers.

RIDDOR-The Reporting of Injuries, Diseases & Dangerous Occurrences Regulations 1995
Employers should report any such cases to the HSE Incident Contact Centre. This includes loss of sight, amputation, fracture and electric shock. In all cases where a personal injury of any type occurs, it should be recorded in an accident book.

The Regulatory Reform (Fire Safety) 2005
All premises must have adequate means of dealing with a fire, and all members of staff should know where these are. This can include fire extinguishers and blankets; however, you should only operate a fire extinguisher if you have been properly trained to do so. All equipment should be checked and maintained regularly.
Fire Drill notices should be clearly displayed and should inform people of what to do in case of a fire. All staff should be trained in the location of alarms, exits and meeting points.

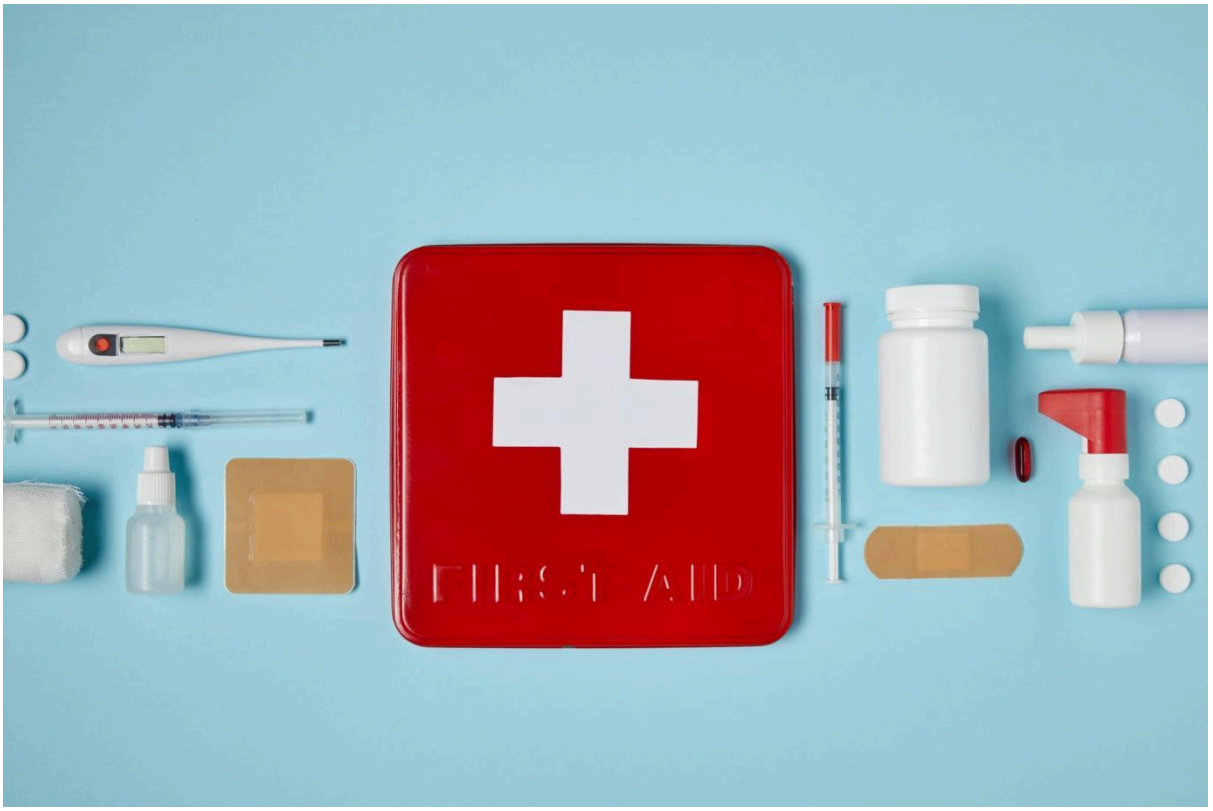
Consumer Protection Act 1987
This Act aims to protect the customer from unsafe or defective services or products. All staff should be trained in using and maintaining products.

The Provision and Use of Work Equipment Regulations 1998
This states the duties of any users of the equipment. It identifies the requirements in selecting and maintaining suitable equipment, as well as the training and safe use of it.

Cosmetic Products (Safety) Regulations 2008
These regulations require that cosmetics and toiletries are safe for their intended purpose and comply with labelling requirements.

Disability Discrimination Act 1996
These Acts prohibit the use of false descriptions of goods or services. The information must always be accurate, false comparisons must not be made, and misleading price comparisons must not be made. A product may not be described as being of a 'reduced' price if it has not been available at a higher price for a minimum of 28 days.

The Equality Act 2010
gives disabled people important rights of access to everyday services. Service providers have an obligation to make reasonable adjustments to premises or to the way they provide a service. Sometimes it just takes minor changes to make a service accessible. What is considered a reasonable adjustment for a large business such as a bank, may be different from what is a reasonable adjustment for a small local salon. It is about what is practical in the service provider's individual situation and what resources the business may have. They will not be required to make adjustments that are not reasonable because they are unaffordable or impractical.



Reporting Accidents

All accidents and near misses should be recorded in an Accident Report Book, which should be kept with a first aid kit on the premises.

The following information must be recorded:

Full name and address of the person(s) involved in the accident.
Circumstances of the accident.
Date and time of the accident.
All details of what may have contributed to the accident.
The type of injury that occurred and treatment provided on or off-site.
Details of any witnesses.

Personal Protective Equipment (PPE)

During certain procedures, PPE should be worn by not only the therapist but also the client if necessary. Types of PPE include:

Disposable gloves
Disposable Aprons
Facemasks
Face Shield or Goggles
Disposable cape



Salon Hygiene, Health & Safety

The salon should be cleaned thoroughly every day.
The working area must be cleaned before and after every client.
Fresh towels and linen should be used for every new client that has been laundered at a minimum of 60°C.
Couch roll, disposable plastic sheeting or waterproof bed sheets need to be used to protect the couch and keep the area as clean as possible.
Products should be dispensed from purpose-specific pump or spray bottles. Creams can be removed from jars or bottles with clean spatulas.
Replace all lids on products securely after use.
All tools that are non-disposable should be sterilised prior to use.
Bins should be metal and have foot pedal operations and be emptied every day. Bins should be collected by an appropriate commercial waste disposal company.
All fire exits should be clearly marked and accessible at all times.

Read all labels and follow manufactures instructions.
Know the hazardous warning signs on products.
Store products safely and in accordance with safety data sheets.
Ensure equipment is placed on a sturdy surface and cannot fall off.
Check wires and plugs regularly on any electrical equipment. Ensure electrical equipment is PAT tested annually. Faulty equipment should not be used.
A first aid kit that complies with the Health and Safety (First Aid) Regulations 1981.

Appearance of the Therapist

A therapist should ensure that they look well presented at all times. Therapists will be working in close contact with a client, and it is important that a professional image is observed.

A therapist should:

Wear clean, freshly laundered and ironed uniform each day.
Wear clean, flat, closed-toe shoes.
Have short, clean, manicured nails.
Have a fresh breath.
Wear antiperspirant.
Apply modest makeup for a natural look.
Wear hair up and away from the face.
Wear minimal jewellery.

Professional Ethics and Standards of Practice

Therapists should:

Maintain the highest possible standards of professional conduct.
Always be courteous and show respect for clients, colleagues and other professionals.
Never gossip or criticise another therapist, salon or brand.
Never talk across a client to another member of staff.
Not to engage in conversations about politics, religion or race that may cause offence.
Maintain a good reputation by setting an example of good conduct in all your communication with clients, team members and visitors to the business.
Ensure to make the treatment or service special for every client.
Respect client confidentiality.
Explain the treatment to the client and answer any questions and queries prior to carrying out the treatment.
Treat all clients in a professional manner at all times regardless of their race, colour, religion, sexual orientation or ability.
Not to treat minors or clients with limited mental capacities, such as those with Alzheimer's or dementia without prior written consent from a parent or carer.

Practising good ethics is essential for the reputation of the therapist and the welfare of the clients. The following is an example of standards and ethics for therapists:

Conduct yourself in a professional, honest and ethical manner.
Promote professionalism
Establish a treatment plan with your client and evaluate the outcome at the end of every session.
Truthfully represent your credentials, qualifications and education, experience, training and competence relevant to practice.
Maintain the confidentiality of the client.
Take a full medical history of the client and ensure that they are suitable for treatment and the treatment is the best solution for their concerns.
Give full aftercare advice.

Precautions Taken in the Salon to Prevent Contamination and Cross-Infection

Hands
Wash with soap/disinfectant and warm water before and after each client—dry hands with a paper towel or blower.

Surfaces
Wipe over with disinfectants, e.g. Alcohol, Surgical spirits.

Treatment of Wounds
If the skin bruises or bleeds after the insertion of a needle, a small pad of dry cotton wool should be used over the area to cover it and apply pressure until the bleeding stops. Apply aftercare solution to the area and work in a different area. The same applies to extractions or any other form of skin piercing. Use disinfectant to clean area.

Disposal
Sharp metal instruments, e.g. needles, lancets, should be placed in a sharps box after use. When the box is about 3/4 full, it may be disposed of by special arrangement. Usually collected by local health office and incinerated at a local hospital.

Metal Instruments
Sterilised before and after each client in Autoclave or in Glass bead steriliser, and wipe with Chlorhexidine Gluconate or Methylated spirits.

Skin Preparation
Do not use sharp or pointed instruments on or at least near areas of a client's skin that are obviously diseased, infected or inflamed. Except in facial treatments during the extracting phase (a tile with a lancet and cotton wool dampened with methylated spirits and an antiseptic solution containing Chlorhexidine Gluconate must be prepared, hands should be washed before and after extractions and finger cots or gloves must be used).

Cuts on your Hands
Cover existing wounds with a waterproof dressing, wash fresh cuts and encourage bleeding under running water and then cover with a waterproof dressing. Clean with an antiseptic. Always have a box of plasters/waterproof dressing available. No salon should be without a first aid kit.

Needles
Do not test needles on yourself. Test needles on a damp of cotton wool held with tweezers. Needles should only be used once, and needles must not be used on more than one client.

Creams
Tubes are better than jars. Always use a spatula to obtain creams from containers. Never use fingers and always close a container after use. Excess product must not be returned to containers.

Waxing
Whenever blood is drawn on a strip of wax, it must be disposed of. Do not re-use that wax as well as material strips. Pay attention to the following: Hands, lancets, tweezers, surface, disposal, gloves, bin liners, products - wax)

Colds/Flu
Wear a surgical mask. Wash your hands regularly, especially after sneezing or blowing the nose. Also, wash hands in general after touching other surface areas. General advice - stay at home when feeling ill or send employees home if they develop cold/flu symptoms at work.

Waste Bins
Bin liners. Emptied regularly. Bins should have lids.

Gloves
Surgical gloves can be used, e.g. epilation or to prevent contamination. Used always during waxing, brow shaping and any action that may come into touch with blood.

Instruments
Must be cleaned, sanitised and sterilised or where appropriate disposable tools should be used.

Sterilisation Methods

Autoclave



Works like a pressure cooker.
Consists of 2 chambers. Water in the lower chamber and instruments on the upper chamber.
The principle of sterilisation is moist heat.
The water boils in the lower chamber and steam is released towards the upper chamber. Instruments are left in the unit for 10 - 20 min. Afterwards, instruments must be placed in a sterile and clean container.
The moist heat autoclave operates at 121°C and is considered a very effective means of sterilisation.
Other types available, e.g., dry heat autoclave, vacuum autoclave, flash instrument autoclave.

The time and temperature of dry heat autoclave is 160°C (320°F) for 2 hours or 180°C (356°F) for one hour.
Consult manufacturer's instructions and local government laws and regulations on sterilisation times and temperatures.

Advantages of an Autoclave

Economical and very effective
Non-toxic on instruments
Easy to operate

Disadvantages of an Autoclave

Sharp instruments can become blunt.
Metal instruments might rust. Recommend use of stainless-steel instruments.
Expensive
Plastic instruments will be damaged.
Autoclaves will need to be kept clean.
Regular servicing and calibration are required of the device.

Glass Bead Steriliser



Operates at approximately 300°C.
Metal instruments will thus be completely sterilised within minutes.
Only the parts covered with beads will be sterilised.
The unit takes + 20 - 30 minutes to warm up before sterilisation can take place.
If more than one instrument is placed in the container, a longer time must be added for sterilisation.
Consult manufacturers` instructions and local government laws and regulations on sterilisation times and temperatures

Wet Sterilisation (Chemical)

Asepsis can be obtained by washing down all surfaces, walls, floors, treatment beds, tiles, trolleys, work surfaces, basins etc. after basic cleaning with an antiseptic solution. EG: Antiseptic solution concentrates, diluted according to manufacturers' instructions. Towels can also be disinfected in this method. If metal tools are sterilised by this method, the liquid must contain a rust inhibitor.

UV Cabinet

They are used for the maintenance of your sterilisation process.
Basically, used as a storage unit.
They are not used for sterilisation only for sanitation.



This cabinet will keep your item as clean as it was when you first inserted it.

Antiseptics and Disinfectants

Antiseptic

A diluted disinfectant that is safe to apply to the skin. Its' task is to slow down multiplication, growth and in some cases may destroy/kill micro-organisms if the strength of the solution is correct, e.g. some soaps (hands), alcohol and hydrogen peroxide etc.

Disinfectant

A chemical agent which destroys or kills all micro-organisms. Safe to apply on surfaces but too toxic to be applied directly onto the skin, e.g. Quaternary Ammonium compound/Quats, formalin, ethyl or grain alcohol.

Ergonomics

Posture is important, whether you are sitting or standing up to do a treatment. Try to find a working position that is comfortable for you and reduces the need to lean over to just one side.

Using height adjustable treatment couches and chairs. Choose a height that reduces your need for bending over the client. Ideally, your back should be at a 90-degree angle. Your chair should be comfortable to avoid pressure point sores or injury.

Try to avoid twisting the neck, keep your head upright and keep your shoulders relaxed.

Never ignore pain; look at ways to alleviate the symptoms. If you cannot take a break during treatment, then you can adopt gentle stretching techniques.

Repetitive strain injuries can be caused by using the same movements over and over again. Try to avoid repetitive flexing of the wrist and instead alternate by bending elbows or shoulders instead. Equipment should feel comfortable in your hand and have as minimal vibration as possible.

Storage

Make sure you receive a copy of Material Safety Data Sheets (MSDS) from your suppliers.
All staff must be trained on the use of products and equipment.
Training manuals and information leaflets should be accessible to all staff.
Store your products correctly by following the guidance on the MSDS.
Carry out a risk assessment on each product or COSHH report if required.

Keep products in original containers where possible and ensure any decanted products are fully labelled in smaller, purpose-built containers.
Keep all flammable products out of direct sunlight and at room temperature or below.
Mobile therapists must make suitable travel arrangements to avoid spillage and ensure safe working practice and be professional in appearance.

Insurance

There are several types of insurance that are potentially relevant to you as a therapist. The most important is the 'Professional Indemnity Insurance' and 'Public Liability Insurance'. Both of these are necessary in the unlikely event that a client decided to sue you.

Public Liability Insurance - This covers you if a member of the public, i.e. a client or passer-by is injured on your premises or if their personal property is damaged in any way.

Professional Indemnity Insurance - This protects you should a client decide to sue you claiming personal injury or damage as a result of treatments carried out by you.

Employer's Liability Insurance - This is only necessary if you hire others to work for you. This type of insurance would cover you should a member of your staff have an injury on your premises.

Product Liability Insurance - This insurance is important if you plan to use, manufacture or sell products as part of your business. This will protect you in the event that a client is dissatisfied with the product or experiences a reaction to using the product.

Car Insurance - If a car is used for business purposes, ensure that this is covered by the policy and that theft of equipment is included.

Anatomy & Physiology

Skin Anatomy

The skin makes up around 12% of an adult's body weight. The skin has several important functions which include:

S	**Sensation**	The main sensory organ for temperature control, pressure, touch and pain.
H	**Heat Regulation**	The skin helps to regulate the bodies temperature by sweating to cool the body down when it overheats and shivering when the body is cold.

A	**Absorption**	Some creams, essential oils and even much-needed water can be absorbed through the skin.
P	**Protection**	Overexposure to UV light may harm the skin; the skin protects itself by producing a pigment, called melanin, which we see when we tan. Bacteria and germs are also prevented from entering the skin by a protective barrier called the Acid Mantle. This barrier also helps to protect against moisture loss.
E	**Excretion**	Waste products and toxins are eliminated from the body through sweat glands.
S	**Secretion**	Sebum and sweat are secreted onto the skin's surface. The sebum keeps the skin lubricated and soft, and the sweat combines with the sebum to form the acid mantle.
V	**Vitamin D Production**	Absorption of UV rays from the sun helps with the formation of Vitamin D, which is needed by the body for the formation of strong bones and good eyesight.

Skin is made up of 3 major layers known as the Epidermis, Dermis and the Subcutaneous.

The Epidermis

This is the outermost layer of the skin. There are various layers of cells within the epidermis, the outermost of which is called the stratum corneum (or horny layer). The layers can be seen clearly in the diagram of the skin. The surface layer is composed of twenty-five to thirty sub-layers of flattened scale-like cells, that are continually being exfoliated off by friction and replaced by the cells beneath.

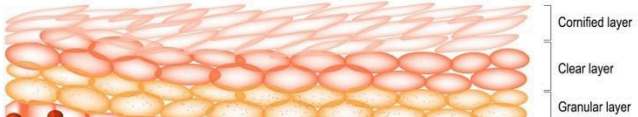
The surface layer is considered the real protective layer of the skin. Cells are called keratinised cells because the living matter within the cell (protoplasm) has changed to form a protein (keratin) which helps to give the skin its protective properties.

New skin cells are formed in the deepest layer of the epidermis. This layer is known as the stratum basale. New cells being to gradually move from this layer towards the stratum corneum to be shed. As they move towards the surface, the cells undergo a process of change from a round, living cell to a flat, hardened cell.

The layers of the epidermis from top to bottom are known as:

Stratum Corneum/Horny Layer
Stratum Lucidum/Clear Layer (only found in the palms on the hands and soles of the feet)
Stratum Granulosum/Granular Layer
Stratum Spinosum/Prickle Cell Layer
Stratum Basale/Basal or Germinative Layer

LAYERS OF EPIDERMIS



Dermis Layer

The dermis is a tough and elastic layer containing white fibrous tissue interlaced with yellow elastic fibres.

The dermis is an expanse layer and contains:

Blood vessels
Lymphatic capillaries and vessels
Sweat glands and their ducts
Sebaceous glands
Sensory nerve endings
The erector pili – which involuntary activates tiny muscles attached to the hair follicle in cold weather to trap heat.
Hair follicles, hair bulbs and hair roots

Subcutaneous Layer

This is the deepest layer of the skin and located beneath the dermis. It connects the dermis to the underlying organs. The subcutaneous layer is mainly composed of loose fibrous connective tissue and fat (adipose) cells interlaced with blood vessels. This layer is generally around 8% thicker in females than in males. The functions of this layer include insulation, storage of lipids, cushioning of the body and temperature regulation.

Hair Anatomy

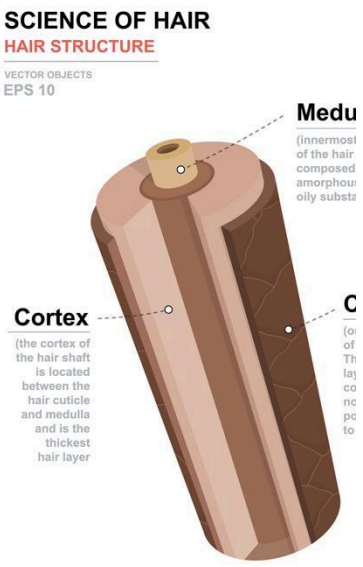
Hair is composed of a strong structural protein called keratin. This is the same kind of protein that makes up the nails and the outer layers of the skin.

Each strand of hair consist of three layers:

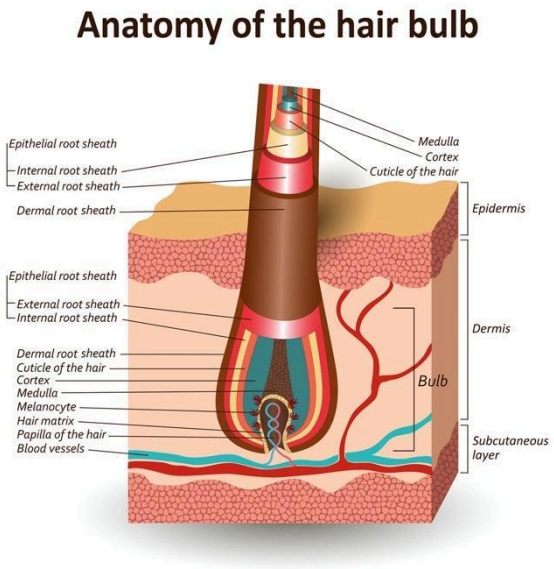
Medulla — An innermost layer only present in large thick hairs

Cortex — The middle layer that provides the hair with its strength, colour and texture.

Cuticle — The outermost layer of the hair, the cuticle is thin and colourless and serves as the protector of the cortex.



Structure of the Hair Root



Below the surface of the skin is the hair root, which is enclosed within a hair follicle.

At the base of the hair follicle is the dermal papilla. The dermal papilla is supplied with nourishment from the bloodstream to produce new hair. The dermal papilla structure is vital to hair growth because it contains receptors for male hormones and androgens.

Androgens regulate hair growth. In scalp hair, the androgens may cause the hair follicle to become progressively

smaller, causing the hair to become finer in individuals who are genetically predisposed to this type of hair loss.

Hair Growth Cycle

Hair follicles grow in repeated cycles. One cycle consists of three phases:

Anagen	Growth Phase
Catagen	Transitional Phase
Telogen	Resting Phase

Each hair passes through the phases independent of neighbouring hairs.

Anagen Phase

Approximately 85% of all hairs are in the growing phase at any time. This growth phase can vary from 2 years to six years. Hair will grow approximately 10cm per year, and any individual hair is unlikely to grow more than one meter long.

In the anagen stage, the hair receives nourishment from the blood supply from the dermal papilla. This enables the cells to reproduce. Cells move upwards to form the different structures of the hair shaft. Melanin is also produced to form the hair colour.

Catagen Phase

This is the transitional phase that follows the growth phase. The catagen phase lasts around one to two weeks. During the catagen phase, the hair follicle shrinks to about 1/6 of the normal length. The lower part is destroyed, and the dermal papilla breaks away to rest below.

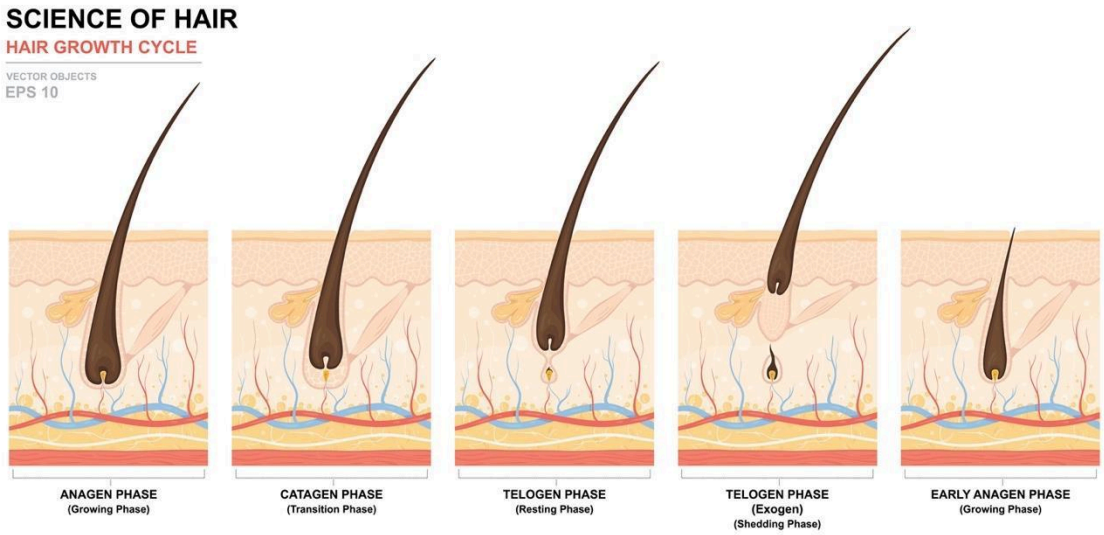
This is the inactive or resting stage of hair growth. In this stage, the dermal papilla breaks away to make the lower end of the hair become loose from the base of the follicle. The hair is still being fed from the follicle wall and is sometimes known as club-ended hair. The hair starts to become drier and continues to move up to just below the sebaceous gland. At this stage, it can be easily brushed out.

Telogen Phase

This resting phase follows the catagen phase and lasts around five to six weeks. During this time, the hair does not grow but stays attached to the follicle whilst the dermal papilla stays in a resting phase below. Around 10-15% of hairs are in this phase at any time.

The hair follicle re-enters the anagen phase. The dermal papilla and the base of the follicle join together again, and a new hair begins to form. If the old hair has not yet

been shed, the new hair pushes the old one out, and the growth cycle starts over again.



Hair Types

Vellus hair is the fine, non-pigmented hair that covers the body of children and adults. It does not usually grow longer than 2cms in length. Vellus hair is usually straight, regardless of ethnic origin, due to the fact that the follicles are not deep. The growth of vellus hair is not affected by hormones.

Terminal hair is the thick pigmented hair found on the scalp, beard, underarms, and pubic area. The growth of terminal hair is influenced by hormones.

Terminal hair is generally more abundant on males than females. However, variations exist within populations with some women appearing hairier than some males.

These hair types can be straight, curly or wavy depending on ethnic origin, hereditary factors and chemical hair treatment such as perms.

European hair would appear to be oval in shape and would tend to be wavy. Asian hair would appear to be round in shape and tend to be straight. Afro-Caribbean hair would appear to be flattened and tend to be very curly.

In some cases, excess hair growth, called hirsutism (pronounced: hur-suh-tih- zum), maybe the result of certain medical conditions. In girls, polycystic ovary syndrome and other hormonal disorders can cause dark, coarse hair to grow on the face, especially the upper lip, and on the arms, chest, and legs. Some medications, like anabolic steroids, can also cause hirsutism.

Superfluous hair is coarse unwanted hair, also termed hypertrichosis. Superfluous hair is considered not only a physical burden but can become a psychological one, too if not treated. The modern woman is keenly aware of the importance of being

well-groomed; her happiness, poise, even her success or failure in life very much depend upon the face she presents to the world.

The reluctance and timidity of women to discuss this condition would vanish if they realised superfluous hair is not something to be ashamed of, but a simple problem requiring remedial treatment.

It is estimated that 80% of the women in Britain have some superfluous hair growth. Generally considered a "cosmetic" issue, hair growth can cause real or perceived problems with social acceptance for many people, both male and female. Many cultures have a perceived ideal amount of hair growth. Changes in hair growth patterns are sometimes a symptom of hormonal imbalances. Managing or removing unwanted hair can go beyond being a simple "cosmetic" problem.

Changes in Growth Patterns

The hair growth pattern can change due to several factors:

Illness - Illness has a strong effect on hair growth, usually making it lank and lifeless. It can also cause hair loss, or growth of unwanted hair.

Medication - Drugs can affect the growth pattern; hair may become coarse and thick. Follicles can often be weakened, causing hair to fall out. In some cases, this can be temporary.

Hormones - Different changes of life can affect hair growth, e.g. women going through the menopause may find they develop whiskers of coarse hair on their face. Endometriosis or Polycystic Ovaries can cause male pattern hair growth in women.

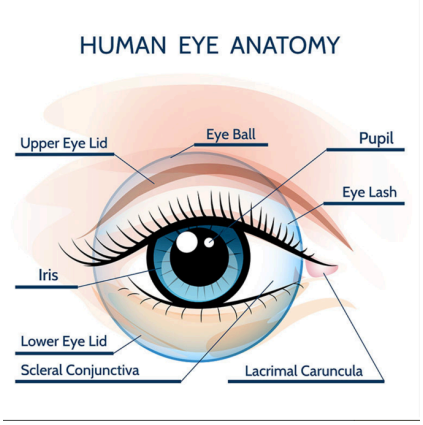
Emotion - Sudden shock, an accident, or stress can all lead to hair loss. This is called alopecia (patches of hair loss).

Anatomy of the Eye

The eyeball is protected and covered anteriorly by two thin and movable eyelids known as palpebrae. The eyeball itself is also covered by a transparent mucous membrane called the conjunctiva, which is continuous along the inner surface of both of the eyelids.

At the medial angle of the eye a small piece of skin called the caruncula lacrimalis is located that contains both sebaceous and sweat glands.

The pupil is the circular opening in the iris. The size of this opening is controlled by the nervous system: at rest, the parasympathetic nervous system constricts the pupil and in danger, the sympathetic nervous system which supplies the pupillary dilator muscle to enlarge the pupil.



The eye is protected and lined by eyelashes. There are often 2-5 rows of lashes and around 100-160 lashes on each row of the upper eyelid and around 50-75 on

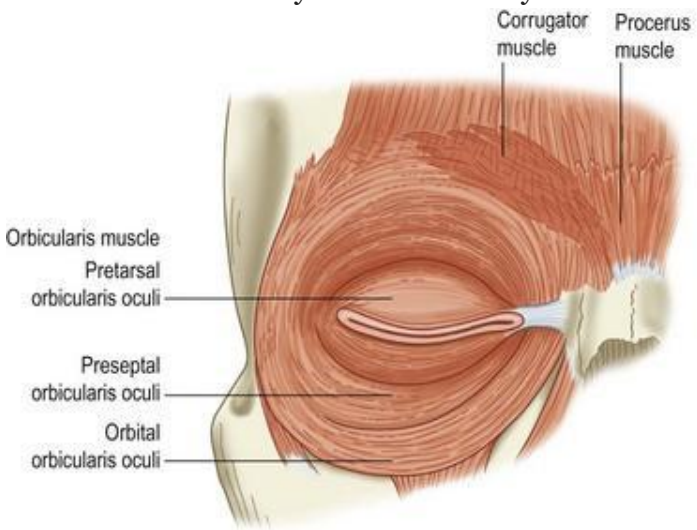
each row of the lower lid. The daily growth rate of a lash is 0.12-0.14mm. The anagen phase varies between four to ten weeks and the complete life cycle of a lash hair is around four to seven months.

The eyebrows function is to protect the eyes from sweat and sunlight.

There are four muscles responsible for facial expression and control of eyebrow position.

These are:

Frontalis	Elevates the brows
Orbicularis oculi	Closes the eyelids and draws the brows towards the eyes
Corrugator supercilia	Pulls the nasal eyebrow inferiorly and medially, producing vertical glabellar wrinkles
Procerus	Pulls the eyebrow inferiorly



The eyebrow growth cycle can last 12-15 weeks. The anagen phase lasts around 30-45 days. The telogen phase lasts for around 2-4 months.

Reasons for having Lash & Brow Treatments

The main function of the eyebrow and eyelashes is to prevent sweat and other debris from entering the eyes. Our eyes are also essential to express our emotions. Despite the practical reasons for our brows and lashes, they have become a vital role in fashion and cultural trends and been the main fixation for thousands of years.



Eyebrows were as significant as far back as Ancient Egypt, where men and women both wore makeup and partook in the current trends. Thick, bold brows were the style of choice for the purpose of paying homage to the God of protection and good health, Horus. The deity is depicted wearing
dark eyebrows, and eye makeup and the Egyptians followed suit and used black oxide and carbon paint around their eyes. It was also believed that this look was though to repel flies and ward off infection, while protecting the eyes from the sun, at the same time as channelling Horus' protective powers. Men used to style their lashes in the same way as women. They used kohl and ointments (which was thought to be an aphrodisiac) to darken the lashes.

During the time of the Greeks and Romans, makeup reflected cultural values that focused on a woman's purity and natural features. In Greece, married women left their eyebrows untouched while unwed women would darken their brows with black incense. The ancient Romans believed unibrows were a sign of intelligence and women even went so far as to draw in a faux unibrow to give the appearance of one.

In the Roman Empire, to enhance eyelashes was a feminine feature. Women were helped by female servants or slaves called ornatrices, who dealt with their beauty. Roman women lashes should be long, thick and curly, as an indication of magnificence brought by the East, from Egypt and India. Plinius the Elder wrote that: "eyelashes fell out from excessive sex and so it was especially important for women to keep their eyelashes long to prove their chastity".

Romans used kohl to darken the eyelashes, and sometimes they added saffron or antimony. They also used burnt cork to thicken and darken their lashes. Small ivory sticks were used as applicants. Things changed with the arrival of Christendom, things. Christian women tried to avoid cosmetics, in the belief that the natural appearance was more pleasant for God, and luxury and cosmetics were generally not accepted.

During the Medieval period, and even in the Renaissance nobody styled their eyelashes. The forehead was considered the most beautiful part of the face, so women often removed eyelashes and eyebrows.

Women of the Tang Dynasty in China typically shaved off their natural eyebrows to allow for an easier application of full-face makeup. They would then apply a bluish-black pigment made from charred willow and draw creative brow styles with tails that drooped. This was soon adopted by the Japanese court not long after. Hair was vital for both men and women in the telling of one's social status. Japanese

noblewomen began shaving off their brows and pencilling on new ones higher on the forehead in an effort to make it easier to apply Oshiroi, a white powder foundation used at the time.



In the Elizabethan times, elevated brows were all the rage, where the higher the brow, the higher the social class. During this era, it was believed that the high hairlines were a sign of aristocracy. Women, therefore, plucked their eyebrows thinner and removed hair from their hairlines to create the illusion of a larger forehead. Walnut oil was often rubbed into hairlines and on eyebrows to thwart growth, even performing this beauty practice on children.

When Elizabeth took the throne, a new fashion era began. Elizabeth had a reddish-gold hair colour, and many ladies dyed their hair and eyelashes with the same shade. However, they used very toxic materials, causing a lot of hair loss.

Enhancing eyelashes was not a very acceptable practise for respectable ladies; they found other approaches to darken their eyelashes secretly, with crushed berries or soot obtained from fireplaces.

Whilst transparent eyebrows had proved popular in previous eras, at the start of the 18th Century, a dark, full brow became desirable. Overplucking and the use of toxic lead-based cosmetics had led to hair loss, making it difficult for women to regrow their brows. As early as 1703, it had become the custom to trap mice and use their fur to form artificial brows as a solution for hair loss. Mice pelts were trimmed into a desirable shape and stuck to the forehead using an adhesive.

During the Romanticism and the Victorian Era, the use of cosmetics between women started to be popular. Old homemade methods were still commonly used.

The personal perfumer of Queen Victoria, Eugène Rimmel developed the first mascara. Since the product was non-toxic, it became popular, very fast.

A newspaper article from 1899 proves that back then women had eyelashes implanted with needles.

In 1911, Anna Taylor, a Canadian woman, first patented artificial lashes. In 1916 the Hollywood director David W. Griffith wanted his actress to have fluttering lashes, so he ordered his film's wig-maker to glue lashes made of a human hair onto actress' own eyelids using spirit gum. This method was not very successful, and fake eyelashes didn't become very popular until the '30.

In 1917, a man Tom Lyle was inspired by his sister, Mabel Williams, while looking at her applying ointment to her eyes to darken her eyelashes. He started working with a drug seller to improve the formula. The final product was "Lash-Brow-Ine," which formula contains petroleum jelly and oils to provide sheen. In 1920 the name was changed to "Maybelline."

After decades of minimal effects on the eyebrow, the cinema and films became a popular form of entertainment in the 1920s and ’30s. The emergence of the movie star began to sway trends in fashion, cosmetics and the enhancement of brows and lashes. These film stars popularised ultra-thin brows by plucking or shaving off the hair, mimicking practices from the Elizabethan times and Medieval Asia. The chic look of the 1920s was pencilling on thin eyebrows with a downward slope creating an intentionally moody look. It was also common to apply Vaseline on their eyebrows to create shine in an effort to draw attention.



Around the same time, the first eyelash curler was invented. These devices were popular and affordable. Women used to heat them with hairdryers. Fake eyelashes also became popular after Vogue published many advertisements featuring more artful fake lashes, but it would be a few more years before they finally became a trend.

The start of World War II in 1939 had a widespread effect on the beauty and fashion industry trends as women were needed to join the workforce and had less time for grooming. Low-maintenance brows that were easy to upkeep became popular. Movie stars still had a relevant influence as the likes of Audrey Hepburn, and Marilyn Monroe sported dramatically arched brows.

As the war came to an end and the decade progressed, brows once again were well-groomed. At this same time, more and more women started using makeup as a daily routine. Painting cat eyes on the upper lashline and cherry red lipstick were fashionable.

At that time the polish immigrant Max Faktor invented the waterproof mascara. These mascaras became very popular and available to the public.

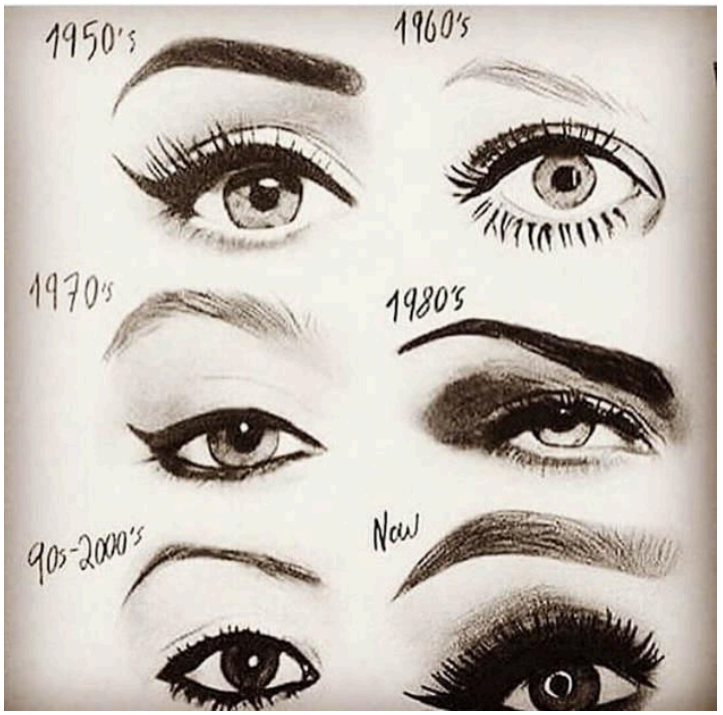
In 1958, Revlon released a mascara in a tube, with a spiral wand, which we continue to use today.

The swinging 60’s brought a range of experimental makeup trends to go alongside the free-spirited fashion of the time. Sophia Loren had the most talked about and coveted brows and was known for shaving off the eyebrow entirely and pencilling them back on in short strokes. Natural makeup looks were common for women emulating the style of celebrities, whilst hippies of the time rejected makeup altogether.

Disco music and the burgeoning nightlife brought about some experimental looks such as dark painted brows and platinum blond hair or thin 20’s era brows tapping into nostalgic glamour. At this time, eyebrow waxing became a widespread treatment in salons and at home kits to maintain the brow.

In 1961, Revlon released the first coloured mascara. Many women followed the famous model Twiggy and started giving more attention to the lower lashes. Fake lashes also trended, made from human hair or synthetic materials and requested by women in every beauty salon. The faker they looked, the better.

The ’80s and ’90s are placed great significance on the brow. During the ’80s the motto of bigger is better saw nails, hair and even shoulder pads take on this challenge. Brows were big and beautiful. The ‘heroin chic’ trend of the 90’s meant brows were groomed but natural and left to frame the face.



By the end of the decade, super thin eyebrows were once again in-trend, and people began over-lucking brows and pencilling them in to mimic celebrities of the time. The 1920’s brow was once again back in fashion. Fake lashes were not fashionable, but coloured mascara became popular and was even used to paint colourful stripes in their hair as well as for their lashes.

At the turn of the Century, brows were a continuation of the thin arches from the ’90s. Brows remained thin. The tweezers were finally laid to rest in the 2010s where people tried to regrow their eyebrows and the damages from decades of over-plucking. The full-grown brow made a come back with the centre tuffs of hair brushed up, giving the look of eyebrows set fairly close together.

The years of brow neglect left to the rise in demand for in-salon brow procedures, such as microblading, lamination and regrowth serums. The rise of social media has now led to the ‘brows on fleek’ movement where women carefully draw, powder and highlight their brows to perfection.

In 2002, an international survey reported that more than 60% of women in the world were using mascara; therefore, this product is 50% of the total amount of cosmetic sales in the world. As of 2009, the mascara market was valued at $ 4.1 billion globally. Women will often choose tinting to enhance their brows, especially if they go to the gym, are busy mums or will be going on holiday.

The lash and brow industry has stood the test of time and will be the bread and butter treatment of salons worldwide.

Contra-indications

A contraindication is the presence of a condition which may make the client unsuitable for the treatment. The treatment may not be able to take place, or the treatment will need to be adapted.

When treating a client, if they show any signs of contra-indications, you should tactfully refer them to their GP for treatment or advice. You should never make a diagnosis even if you are certain of the condition as you may be wrong.

If you are unsure about any contra-indications, then do not treat the client and refer them to their GP.

Be careful if you deal with a contra-indication, and they can often be contagious. Make sure you clean the work area and any implements between clients to prevent cross-infection.

Common contra-indications

General Health & Wellness

Condition	*Description*	*Salon Treatment*
Diabetes	Diabetes is a lifelong condition that causes a person's blood sugar level to become too high. There are two main types of diabetes: **type 1 diabetes** – where the body's immune system attacks and destroys the cells that produce insulin **type 2 diabetes** – where the body does not produce enough insulin or the body's cells do not react to insulin. Type 2 diabetes is far more common than type 1. In the UK, around 90% of all adults with diabetes have type 2. The amount of sugar in the blood is controlled by a hormone called insulin, which is produced by the pancreas (a gland behind the stomach). When food is digested and enters the bloodstream, insulin moves glucose out of the blood and into cells, where it's broken down to produce energy. However, with diabetes, the body is unable to break down glucose into energy. This is because there's either not enough insulin to move the glucose, or the insulin produced does not work properly.	Can often be treated if the client is managing their diabetes well. Check with your insurer as their criteria may not allow treatment. A GP note may be required. Treatments that cause injury to the skin can increase the risk of infection. Those with diabetes will need to understand wound management and treatments should be done under extreme caution as to not injure the skin if necessary. Diabetics have a slower wound healing response and have a higher risk of infection. All products used must be sterile, and care should be taken not to injure the skin.

Epilepsy	Epilepsy is diagnosed when a person has had more than one epileptic seizure and could have more in the future. Electrical activity is happening in our brain all the time. A seizure happens when there is a sudden burst of intense electrical activity. This is often referred to as epileptic activity. This intense electrical activity causes a temporary disruption to the way the brain normally works, meaning that the brain's messages become mixed up. The result is an epileptic seizure. There are many different types of seizure, and each person will experience epilepsy in a way that is unique to them. Some things make seizures more likely for some people with epilepsy. These are often referred to as 'triggers. Triggers are things like stress, not sleeping well and drinking too much alcohol. Some people say they have more seizures if they miss meals. Not taking epilepsy medicine is another common trigger. A very small number of people with epilepsy have seizures triggered by lights that flash or flicker. Avoiding triggers can stop them from having seizures.	If well-managed treatments may be able to be undertaken. It is worth assessing the client and finding out what triggers a fit and when as well as how it is managed and their last episode. You may also need to check your insurer's terms and refer the client to a GP for a letter of approval.
Heart conditions	Heart disease describes a range of conditions that affect the heart. Heart diseases include: Blood vessel disease, such as coronary artery disease Heart rhythm problems (arrhythmias) Heart defects you're born with (congenital heart defects) Heart valve disease Disease of the heart muscle Heart infection Risk of fainting, risk of slow healing or chances that the client may be medicated to keep their condition under control are some risks you will need to be aware of when treating the client. Clients may also have a heart catheter or a pacemaker.	Treatments that do not involve electrical currents being passed through the body are usually safe. Heart conditions should be well managed. You will need to check the terms of your insurance policy. A referral to a GP may be required.

Deficient Immune System	Immunodeficiency disorders prevent the body from fighting infections and diseases. This type of disorder makes it easier to catch viruses and bacterial infections. Immunodeficiency disorders are either congenital or acquired. A congenital, or primary disorder is one you were born with. Acquired, or secondary, disorders you get later in life. Acquired disorders are more common than congenital disorders. Your immune system includes the following organs: spleen tonsils bone marrow lymph nodes These organs make and release lymphocytes. These are white blood cells that fight invaders cells called antigens. Cells release antibodies specific to the disease the body detects. White blood cells destroy foreign or abnormal cells. Examples of antigens that white cells might need to fight off include: bacteria viruses cancer cells parasites An immunodeficiency disorder disrupts your body's ability to defend itself against these antigens.	A deficient immune system can increase the risk of infection. Treatments that break the skin may increase the risk of infection. Clients may need a GP referral and understand wound management.
Pregnancy	Pregnancy is a period of considerable changes in a woman's body. These changes, affecting virtually every part of the body, are all geared towards growing and delivering a healthy baby, without harming the mother. Changes begin within days of conception when the fertilised egg implants itself in the wall of the uterus. The first changes are subtle, and most women will not notice them.	Nearly all treatments are contraindicated in the first trimester. Treatments that cause friction may have a side effect of PIH and should not be performed. Electrical treatments must not be performed at any point.

	Pregnancy lasts an average of 266 days (38 weeks) from the date of conception or 40 weeks from the first day of the last menstrual period. Pregnancy is divided into three periods of three months each. These periods are known as the first, second and third trimesters. Each trimester brings with its own unique set of experiences. Risks of pregnancy include: Miscarriage Increased/decreased blood flow Risk of fainting Oedema Pigmentation/Chloasma/mask of pregnancy Pelvic trauma	
Breastfeeding	Anything the mother consumes or applies to her skin could be absorbed into the bloodstream and through into the milk ducts, where this can pass to the baby. Great care should be taken with what is being applied to the skin as well as avoiding treatments that could cause open wounds. An open wound is susceptible to infection, and this can cause infection or septicaemia. During breastfeeding, hormones are still active within the body, and these can have an overall effect on the skin. This may lead to dehydrated skin (if the mother is not ingesting enough water) and also lead to a high risk of pigmentation. Invasive procedures should be avoided until a few months after the mother has stopped producing milk.	Treatments that break the skin are strictly contraindicated. The client also has a high amount of hormone imbalance still and may be susceptible to PIH on certain treatments.
Electrical implants or pacemakers	A small battery-operated device called a pacemaker is placed into the chest. It sends regular electrical impulses, which help keep the heart beating regularly. The pacemaker is a small metal box weighing 20–50g. It is attached to one or more wires, known as pacing leads, that run to the heart.	Electrical implants contra-indicate any treatment that involves an electrical current. Other treatments that may affect blood pressure may also contra-indicate a procedure. Check your insurer's terms and

	If the pacemaker senses that the heart has missed a beat or is beating too slowly, it sends signals at a steady rate. If it senses that the heart is beating normally by itself, it does not send out any signals. Most pacemakers have a special sensor that recognises body movement or the breathing rate. This allows them to speed up the discharge rate when active. An implantable cardioverter-defibrillator (ICD) is a device similar to a pacemaker. An ICD delivers an electrical shock to the heart during a life-threatening heart rhythm. The aim is to 'reboot' the heart to get it back into a normal rhythm again. Some modern devices contain both a pacemaker and an ICD.	refer the client to GP for a letter of approval.
Anticoagulant medicines such as warfarin or aspirin	Anticoagulants are medicines that help prevent blood clots. They're given to people at a high risk of getting clots, to reduce their chances of developing serious conditions such as strokes and heart attacks. A blood clot is a seal created by the blood to stop bleeding from wounds. While they're useful in stopping bleeding, they can block blood vessels and stop blood flowing to organs such as the brain, heart or lungs if they form in the wrong place. Anticoagulants work by interrupting the process involved in the formation of blood clots. They're sometimes called "blood-thinning" medicines, although they don't actually make the blood thinner. Although they're used for similar purposes, anticoagulants are different from antiplatelet medicines, such as low-dose aspirin and clopidogrel.	Anticoagulants prevent the blood from clotting and can affect the healing process of the skin. The client will be at higher risk of bruising also. Treatments that injure the skin or cause trauma will need to be avoided. A GP letter may be required for some services.
Steroids, antidepressants,	Many drugs can affect the skin. For example, prescription antidepressants, especially the popular class of	Medications can have numerous effects on the skin. If you are unsure,

antibiotics and other medications	medications called selective serotonin reuptake inhibitors (SSRIs), can cause a wide variety of adverse skin reactions, including bruising. Acne medications and certain antibiotics may make the skin more sensitive to sunlight and light or heat-based procedures. It is vital that we find out the medications both prescription and over the counter drugs that the client is taking. The internet is a great source of information to establish the risks the medication may have on the skin during a procedure.	you should ascertain how the treatment affects the skin and the effects of the medication that is being taken. A patch test may be required or a letter from the clients GP. Where the side effects may be photosensitivity or thinning of the skin, then treatments should be avoided until the client has come off their medication for three months at their GPs request.
Recent surgery	Clients will have surgery for a variety of reasons and on any area of the body. Surgery puts the body under extreme pressure. The client may be on pain medication which could affect their susceptibility to feeling pain during treatment. Surgery wounds can take many months to fully heal. Post-surgery complications can also arise a few months after the procedure. This can include, infection or necrosis of the wound, infection of the blood, blood clots, internal bleeding, or fever.	If the client has had recent surgery, you will need to weigh up the risk of the procedure, the length of time between the surgery and treatment as well as where the surgery was. In most cases, it is advised to wait six months post-surgery before undertaking any procedures, especially where these may involve invasive techniques, massage or electrical machines.
Metal plates or pins	If you break a bone, it will need to be held in place while it heals. This can be done using a splint, sling, brace or cast. But sometimes a broken bone will need to be fixed with surgery, using metal plates, rods, wires, screws or pins. Implanted metal can help broken bones heal in proper alignment. While these implants do not help the bone heal faster, they can help to hold bones in the proper position while healing takes place. Implants may include metal plates and screws, pins, and intramedullary rods inserted into the cavity of a bone.	Electrical treatments are almost always contraindicated for clients with metal implants in their body. However, this can vary depending on the area of the implant vs the area of treatment. Care may also need to be taken when massaging over the area or any pressure is applied.

Chemotherapy/ radiotherapy	Chemotherapy is a form of cancer treatment where a patient is given drugs designed to kill cancer cells. Radiation is a type of cancer treatment where high doses of radiation are delivered to cancerous tumours in the body. Both procedures can affect the sensitivity of the skin, and the skin texture may change. A waxy appearance to the skin can alter the effects of treatments and treatments may not be suitable. Clients may also be at risk of pigmentation.	Clients undergoing treatment for cancer may benefit from a number of procedures in a salon for the purpose of relaxation and pain management as well as to make them feel good. Some treatments may be too much for the client to handle and cause unwanted side-effects. It is worth discussing the treatment with the client and working out alternatives where suitable. If unsure, you may wish to ask for a letter from the client's oncologist.
Broken bones	When a bone has an outside force exerted upon it, like a blow or a fall, there is potential that it cannot withstand the amount of force and it breaks. That loss of integrity results in a fracture. It is important to remember that a fracture, break, or crack all describe the same situation, an injury to the bone where it has been damaged. One term is not more serious than another. Fracture break and crack all mean the same thing.	Clients with a suspected or recent fracture should be treated with caution. If working over the area of a recent break, then the time between the break and the treatment should be at least six months. If working on other areas of the body, it is best to wait for 6-8 weeks post-injury as the client may be more susceptible to an embolism.
Previous or recent treatments in the area	When providing a service to the client, it is important to establish what previous treatments they have had, not only at clinics but at home. You should look to enquire over the products they use regularly that may affect the skin and the treatment outcome.	Some treatments can affect the skin by either removing out layers of the stratum corneum or stimulating a healing response. You will need to identify if the client has had any recent procedures in the treatment area you will be working on.

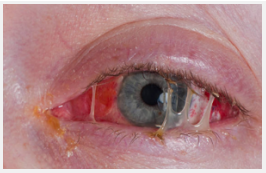
Contra-indications

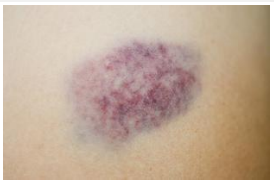
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CONDITION	IMAGE	DESCRIPTION & CAUSE	SALON TREATMENT
PSORIASIS		Scaling and inflammation of the skin. Cause unknown but thought to be related to the nervous system	Treat with caution; avoid the affected area. Do not treat if the skin is weeping.
ECZEMA		Atopic eczema is a common skin condition that causes patches of skin that are itchy, cracked and sore.	Treat with caution: avoid the affected area.
CONJUNCTIVITIS		A transparent and sticky substance covers the white of the eye and lids. It is caused by a bacterial infection.	This is highly infections; do not treat; the client should be referred to their GP for correct diagnosis and treatment.

CUTS & ABRASIONS		Broken skin caused by an injury.	Avoid treatment in the affected area.
BACTERIAL KERATITIS		A severe disorder which can result in partial or total loss of vision. It is caused by a bacterial infection.	Do not treat; refer clients to their GP for correct diagnosis and treatment.
BRUISING		Black, green. Yellow or red marks appear on the skin. They are generally caused by an injury.	Avoid the area if possible.
RECENT OPERATIONS (SCARS)		Scar tissue raised or flat undergoing the healing process. Scar tissue is very sensitive.	Avoid treatment if the scar is less than six months old.
BLEPHARITIS		Inflammation of the eyelids; the inflammation is like eczema of the skin with red, scaly eyelids; you may notice tired or gritty eyes, which may be uncomfortable in sunlight or smoky atmospheres; they may be red and swollen and feel as though there is something in them. The exact cause is unknown, but people	Avoid the area; refer the client to GP for correct diagnosis and treatment.

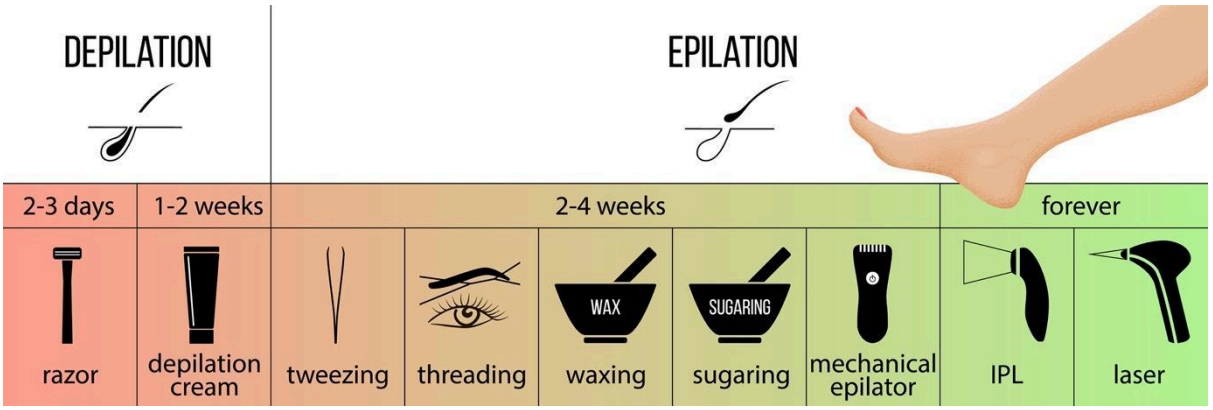
		who have dandruff or dry skin conditions may be more prone to blepharitis.	
STYE		Infection in the root of an eyelash. They are caused by a bacterial infection.	Avoid the area; no treatment until the infection has gone.
SUNBURN		Sunburn is a red, painful skin that feels hot to the touch. It usually appears within a few hours after too much exposure to ultraviolet (UV) light from sunshine or artificial sources, such as sunbeds.	No treatment should be provided until the skin has healed. 48 hours should be left from sun exposure/sunbed use before a treatment.
COLD SORES		Cold sores are painful lumps or blisters on the face. They're caused by a virus and are very contagious.	Treatment should not be provided on the face until the skin has healed. Precautions should be taken when treating other areas to avoid cross-contamination.
IMPETIGO		Impetigo is a highly contagious skin infection that often starts with red sores on the face or hands. It causes red sores or blisters that burst and leave crusty, golden-brown patches.	Treatment should not be provided on the face until the skin has healed. Precautions should be taken when treating other areas to avoid

			cross-contamination.
MOLES		*Moles* are small, coloured spots on the *skin*. Most people have them, and they're usually nothing to worry about unless they change size, shape or colour.	Avoid treating over a mole. Irritation or damage to the area may cause the mole to change.
RINGWORM		Ringworm is a fungal infection on the skin. It causes a rash that is often ring-shaped.	Treatment should not be provided to the client until the condition has been treated.
SENSITIVE SKIN		Sensitive skin is a common issue but not a medical diagnosis in itself. The term generally refers to skin that is more prone to inflammation or adverse reactions. People with sensitive skin may have strong reactions to chemicals, dyes, and fragrances present in products that come into contact with the skin.	Treatments should be applied with caution. Patch tests may be required before undertaking a full procedure.

Methods of Hair Removal

Method	*Pros*	*Cons*
Shaving	Fast Instant	Temporary only lasts a day or two. Can cause ingrown hairs. Chance of cuts from the razors.
Hair Removal Creams	Fast	Overpowering Smell

	Instant No chances of cuts Smoother Skin Lasts longer than Shaving.	Irritant to the skin. Not always 100% effective. Cannot be used on sensitive or intimate areas. Can cause ingrown hairs. Can cause skin Rashes or Sensitivity.
Plucking/Tweezing	No chance of cuts. Great to target individual hairs. Precision Lasts a couple of weeks.	Time-consuming/not good for large areas. Causes more discomfort for a longer period of time.
Epilation Device	Removes hair from the root, so hair removal lasts a couple of weeks. No chance of Cuts. No Skin irritation. Hair grows back softer and finer.	Cannot be used on small areas like the face. Can be uncomfortable. Especially painful on more sensitive areas like underarms and bikinis. Time Consuming.
Hair Mitts	Skin is left soft and hair-free.	Only lasts a couple of days. Can cause skin irritation.
Bleaching	Disguises unwanted darker hair. No pain if used correctly.	Skin can become sensitive. Only lasts a week. Hair is still present.
Electrolysis	Permanent hair removal.	Can be uncomfortable. Time-consuming. Costly. Hair has to be present for treatment to commence.
Intense Pulsed Light	Permanent Hair Reduction. Painless. Can shave in-between treatments. Hair loss is achieved within 6-9 Sessions. No more ingrown hairs. Great for intimate areas.	Costly. Cannot tan between treatments. Can cause pigmentation changes in the skin. Does not work on white or blonde hair.
Waxing	Lasts three to four weeks. Can be used on any area of the body. Fast. Affordable. Can reduce ingrown hairs or itching. Hair regrowth is softer, finer or ceases altogether.	Can be painful depending on the area treated. Possibility of skin trauma such as bruising or burns. Needs to be done every few weeks. Hair needs to be a certain length before it can be waxed.



Contra-actions

Contra-actions are reactions of a client caused by a treatment taking place.

You will need to explain to your client what/if any reactions to expect during or after treatment.

With all contra-actions tell your client that if they do not see any improvement within 24 hours to get in touch with their GP for advice.

CONDITION	DESCRIPTION AND CAUSE	COURSE OF ACTION
SLIGHT SWELLING	Tender puffy skin. When working on more delicate skin.	Apply cold compress
BLOOD SPOTTING	Tiny blood spots appear when strong hair has been removed.	Apply pressure and wipe over the area with antiseptic
ERYTHEMA	Reddening of the skin due to dilation of the blood vessels. A reaction to external stimulus or infection.	Apply cold compress
EYE WATERING	Excessive watering of the eye. A natural reaction as the eye tries to remove any irritant.	Should settle at the end of the treatment.
STINGING/BURNING EYES	Stinging or burning sensation of the eye, followed by reddening of the eyeball. Tint or product enters the eye, causing irritation to the cornea.	Remove the product properly. Give client damp cotton wool and sit them up to wipe over their own eyes. If stinging persists, flush the eye with saline. If this doesn't settle

		the irritation, the client made need to be referred to hospital.
ALLERGIC REACTION	Stinging, burning, itching, hives and reddening of the skin. Allergy to any of the products used.	Remove the product immediately and wipe over the area with damp cotton wool. A cold compress may be used. The client will need to be referred to hospital.
BURNS	Redness, tenderness, skin peeling or discomfort during or post-wax.	Depending on the severity of the burn. Wax should be removed immediately with an after-wax lotion or oil. A cold compress applied to the area. The burn is usually quick to heal, but the client may wish to seek medical attention.
FOLLICULITIS	An inflammation affecting the hair follicles, which are the hair-producing pores on the skin. Folliculitis is characterised by a rash with clusters of red bumps or whiteheads. Folliculitis is usually caused by bacteria.	The area will need to be exfoliated regularly, and a tea tree lotion applied daily.

Client Consultation

A client consultation is a one to one talk with your client. During this time, you will find out very important and confidential information that will allow you to advise and provide the best treatment for the client.

It is important to always introduce yourself to the client as this removes any barriers and relaxes them. Consultations should always be undertaken in a private room or area where you cannot be overheard by others.

A client should first fill out a client consultation which helps identify any contra-indications that may mean you have to alter the treatment or be unable to treat them at all. If their form shows no reason why they cannot proceed with the treatment, then you can move onto verbal questioning.

Verbal questions would be to establish why the client has visited the salon and what their expectations and outcome of the treatment may be. Asking what they want ensures you can provide customer satisfaction as the client should be pleased with the outcome of their treatment. It is good practice to speak to the client in front of a mirror and explain the treatment to them and see if that meets their requirements.

Once you have established what the client is after, then a physical examination should be undertaken. This allows you to further check for any undeclared contra-indications and get a better overview of any issues that you may face during the procedure.

Allow around 15 minutes for the client's first salon visit. Ideally, you should sit face to face or next to the client to create an open atmosphere. Avoid barriers such as a couch or table between you.

Use open questions to tactfully encourage the client to give you information that you may need rather than using interrogating questioning techniques. Use the consultation form to work from and record anything you may discuss.

Record Keeping

Records must be maintained and updated for a number of reasons.

They provide contact details in case you need to alter or cancel an upcoming appointment.
So that you can track client's progression.
To record the products used and timings so you can use these at further visits and adjust the treatment plan if required.
Tracks any aftercare you provide the client.
Records patch test history.
As a backup in case, the client has an adverse reaction to treatment.
For legal reasons if the client brings a claim against you.

Client records can be stored electronically or filed away manually and should be updated at every visit. If consultation forms are not updated and do not contain a history of services and dates, then you may find your insurance invalid.

Forms should be kept for the timeframe suggested by your insurance company. This may be for up to six years. If a client is under 21 at the time of service, then it is recommended to keep the forms for six years past their 21st birthday.

Client confidentiality must be protected at all times. Forms need to be locked away in a secure cabinet, and electronic records should be held on a password-protected computer. You may also need to register with the ICO as a data controller.

All information must be accurate and necessary for the service or treatment being performed.
Individual client records must be available for the clients to view if requested.
Data should not be passed on or sold without the client's prior written permission.

The following details should be recorded on the client consultation form:

Personal details – name, address, contact details
Results of any patch tests
Contra-indications
Contra-actions
Reasons for the treatment
Any reactions to treatments/previous treatments
Home care advice/suggested retail items.
Any sales
Treatment timings/products used etc.
Next appointment or recommendations

Any contra-indications and possible contra-actions should be identified and discussed prior to the treatment. In the case of a medical referral, the therapist should keep a copy of the GP's letter with the client's record card.

Consultation forms must be signed and dated to prove that you have covered everything and given the correct advice and treatment plan.

Sensitivity Testing

New clients or clients new to waxing should have a sensitivity test. This is to ensure that the client has the ability to feel the difference between hot and cold as well as feel any pain.

This allows the therapist to ensure the client can accurately feel the temperature of the wax and inform the therapist if the wax is too hot, or the treatment too uncomfortable. If the client was unable to feel the heat of the wax, then burning of the skin may occur.

To test the skin for sensitivity, you will require something hot and cold and something sharp and blunt. Apply each at alternative times to the area you wish to treat and ask the client if they feel, hot, cold, sharp or blunt sensations.

On clients new to waxing, a test patch of wax may be suitable so they can make an informed choice on whether to undertake the full treatment and see how the skin may react.

Client Consultation form

Name: Date of Birth:
Address:

Postcode:
Phone Number:
E-Mail:
How did you hear about us?

Please tick any of the below that may apply to you:

Open wounds	( )	Pregnancy	( )	Allergies	( )
Asthma	( )	Verruca's	( )	Eczema	( )
Epilepsy	( )	Cold Sores	( )	Rashes	( )
Psoriasis	( )	Sunburn	( )	Recent Scars	( )
Conjunctivitis	( )	Impetigo	( )	Ringworm	( )
Burns	( )				

Doctor's permission must be sought before treatment if you have ticked any of the above boxes.

Patch test details:

Date of patch test: Product used:
Placement: Outcome of test:
NEGATIVE/POSTIVE

Date of patch test: Product used:
Placement: Outcome of test:
NEGATIVE/POSTIVE

Date of patch test: Product used:
Placement: Outcome of test:
NEGATIVE/POSTIVE

Are you on any medication taken orally or applied topically? If yes, please provide details.

Have you had a recent Skin peel, Microdermabrasion or are you using Glycolic based skincare? Or have you had any recent filler injections?

I confirm that the above information is true to the best of my knowledge and belief. I have been fully informed about the expected results and effects of waxing and agree to follow all the aftercare advice provided by my therapist. I hereby give my consent to proceed with treatment.

Under GDPR rule, I understand that I have full access to all data held on me. This data will be held by the clinic for no longer than six years for insurance purposes, after which, digital information will be deleted permanently, and paper documents will be destroyed. All information on myself is kept on password encrypted hard drives or locked in filing cabinets to which only selective staff members have access. None of my personal data will be sold or used for anything other than to provide the services of this clinic.

Clients
Signature:_____Date_____
_____

Therapists
Name:_____Signature:_____
_____

Date	Treatment Details	Client / Therapist Signature
	Please sign here to confirm the details overleaf are correct and that no changes have taken place since your last treatment.	
	Please sign here to confirm the details overleaf are correct and that no changes have taken place since your last treatment.	
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Area set up

When offering lash and brow treatments, you will need to ensure you have the correct equipment and working conditions to undertake the treatment.

To offer lash and brow treatments, you will need:

Beauty bed or reclining chair for your customer	
Adjustable therapist stool/chair	
Trolley or side unit where you can place everything you need to perform the treatment. This should be fully wipeable and at a decent height for working.	
Swing bin or pedal bin for rubbish under the bed/chair.	

Decent lighting within the treatment room/area. Additional lighting can be used, such as a mag lamp or ring light.	
Hand mirror for the client to hold during the consultation and during/post-treatment to discuss a treatment plan.	
Room for you to work around both sides of the client.	
Chemical sterilising fluid and jar for metal and plastic implements.	
Wax pot set to the correct temperature.	
White kohl or wax pencil and sharpener to mark brow placements or mark out areas that are contra-indicated.	
Tweezers to remove fine hairs leftover by wax	
Scissors for trimming long hair	
Wax, non-strip or strip, depending on the treatment area.	
Pre-Wax & After wax to cleanse the skin prior to waxing and to soothe skin post-treatment. Eye makeup remover, oil-free and suitable for sensitive eyes to remove all traces of makeup and dirt prior to treatment.	
Cotton wool or gauze to remove makeup, product and apply aftercare.	

Small bowls for damp cotton wool or water.	
Couch roll or paper towels to protect clothing	
Gloves – Nitrile, disposable.	
Disposable apron – to protect work clothes	
Wooden spatulas, selection of small and large, for using on different areas.	
Wax Strips – Paper or fabric strips to remove warm wax.	
Selection of lash tint colours: Light Brown, Dark Brown, Black, Blue, Blue-Black, Grey, Red.	
Peroxide to activate the tint.	
Eye wash/Saline pods to rinse out product that may accidently enter the eye.	
Tissues for clients with sensitive, watery eyes, cleaning out tint dishes and placing objects on to keep work surfaces clean.	
Spool brush or disposable mascara wands to brush and tidy hairs into place.	

Under eye protectors prevent staining of the lower eye lid during tinting procedures.	
Tinting dish to mix tint.	
Tint applicators, ideally disposably to apply tint to the clients lash and brow hair.	

Preparing your work Area

Preparation of your work area is essential prior to your client arriving. Ensuring you have everything you need to hand allows the treatment to run smoothly and on time.

Ensure the bed is a good height for waxing. Cover with a plastic sheet to protect the bed and then cover over this with couch roll.
Ensure your wax is on and set to the correct temperature. The wax pot should be on a suitable surface, ideally a trolley so that you can keep it close to you at all times and prevent spillages. Ensure the wax pot is secure so that it cannot 'slip' or 'fall' off. Special wax trolleys have deep lips or bars around the top shelf to prevent items from falling off.
Ensure your trolley is stocked with everything you need, this includes;
Pre-Wax Spray or lotion. This helps to remove any residue of oils or sweat off the skin's surface and acts as an antiseptic.
Vinyl Gloves.
Talcum Powder, good to apply to the skin prior to waxing in sensitive areas. It helps prevent the wax adhere to the skin.
Wax strips, these can be fabric or paper bonded strips. Depending on the area you are treating these should be cut to size.
Pair of scissors soaking in a sanitiser.
Pair of Tweezers soaking in a sanitiser.
Wax remover for cleaning up wax spillages on the trolley or floor.
Couch Roll or Tissues to protect the client’s clothes.
Cotton Wool to apply Pre-Wax treatment.
After wax oil helps dissolve areas of wax still left on the client.
After wax lotion, this helps to calm and soothe the skin after waxing and also breaks down any sticky residue left from the wax.
Tinting kit.
Towels for client modesty.
Disposable Spatulas or orange sticks.
A bin close by preferably a foot-operated one.
Adequate lighting! You cannot wax in poor lighting so ensure your room is well lit.
Disposable Aprons. Not essential but prevent wax spills or oil spills going onto your clean uniform

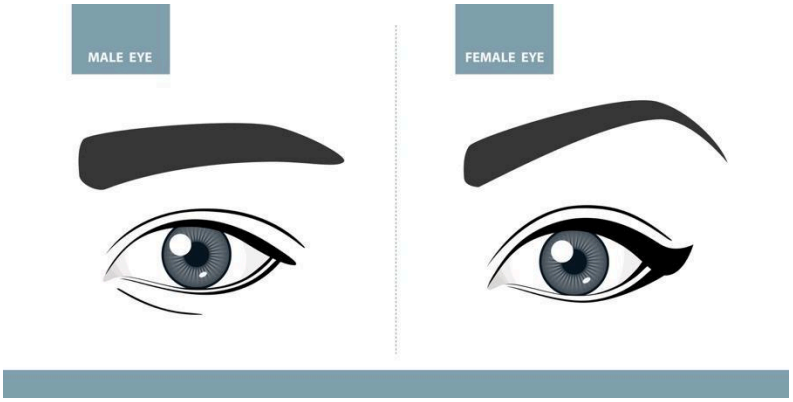
Eyebrow Shaping

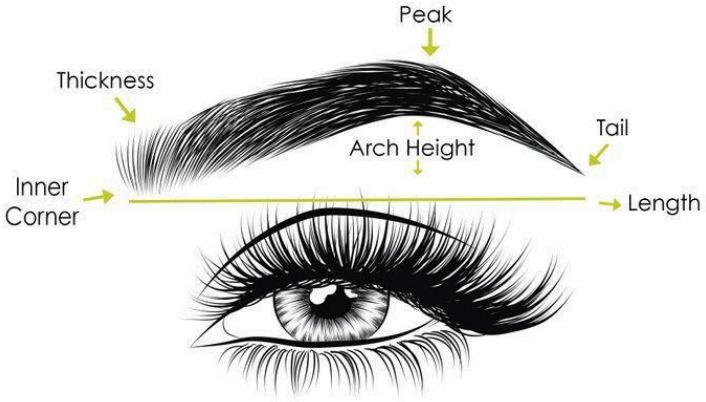
Eyebrow shaping is the removal of excess hair to enhance the shape of the client's natural brow. When shaping the client's brow, you also need to take into account your clients' needs and expectations. Discuss the shape with your client.

Work slowly; checking with your client as you go to make sure that you don't remove too much hair.

The Right Shape

The basic shape of the eyebrows and the direction in which the hairs grow cannot be changed. However, all eyebrows should be shaped so that they are thicker at the head of the brow (the part closest to the nose) and then gradually get thinner.





Everyone's face and brow shape are different; there are five basic brow shapes that can be identified, which can also have a number of variations to them. Each of the five shapes has the ability to change the appearance of other facial features, therefore it is important to choose the right shape for the client.

BROW SHAPE	DESCRIPTION	FACE SHAPE
FLAT BROWS 	These brows are ideal for clients with a long face shape as the horizontal line creates the illusion of a shorter face with a more oval appearance.	
ROUND BROWS 	The round shape of these brows is perfect for trying to create a softness to the face. Perfect for clients with sharp features to help minimise their appearance.	
CURVED BROWS 	Curved brows are completely different to round brow shapes. They are perfect for clients that do not want too much softness to their features and can work really well on square face shapes.	
ANGLED BROWS 	The high arch of angled brows have the ability to create a more youthful appearance and can make round or diamond face shapes appear slimmer.	
SOFT ANGLED BROWS 	Similar to the above but with a much shorter arch and a softer peak. The soft angle is great for a more feminine look and perfect for an oval face.	

Influences affecting the Shape of your Clients Brows

Natural Brow Shape	If the client has shaped her own brows for a long period of time, they may need to let them grow to get their desired shape.
Clients Age	Mature clients may have coarse hairs which can be longer, straight or in different colours.
Fashion Trends	Fashion trends may affect the client's brows permanently. Especially if thin or over-plucked brows become the fashion.

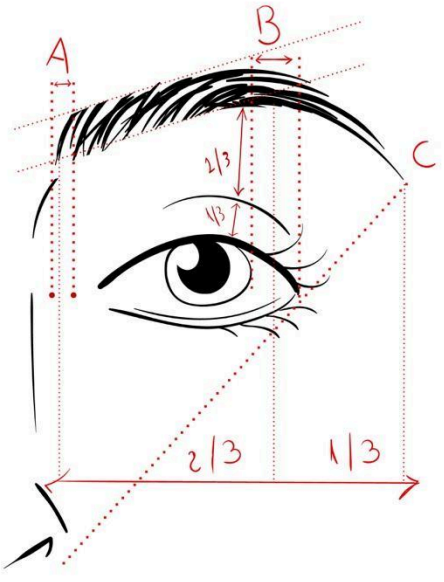
Measuring the Brows

Once a consultation has been carried out, and a shape decided the brows should be measured to create the perfect shape and symmetry

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To measure the brows:

Place an orange stick in a straight line from the side of the nose to the inner corner of the eye. This is where the eyebrows should begin.
Place the orange stick from the side of the nose to the outer corner of the eye, and this is where the eyebrow should end.
Ask the client to look straight ahead and place the orange stick from the side of the nose through the outer cornea part of the eye. This is the highest point of the arch.
Holding the stick vertically so that it runs from the inner brow line to the top of the ear. Draw a line between both the bottom and top of the brow bulb. This ensures the brows are symmetrical.



Lash & brow tinting - patch test

Lash & brow tint contain ingredients that may cause skin irritations or allergic reactions on some individuals. A patch test must be performed at lease 24 hours prior to treatment. This may also be a requirement of your insurer.

How to do a patch test

Use soap and water to cleanse a small area of skin behind the ear or on the inner surface of the forearm.
Follow the preparation instructions for your brand and mix a small amount of tint & developer together.
Apply the tint to the area you have prepared on your client.
The client should leave the tint on their skin for 24 hours and then remove with soap and water.
If there is no irritation or inflammation in the area, then it is assumed that there is no hypersensitivity to the dye.

A negative result on a patch test does not mean that the client will not react during treatment.

Patch tests should be performed regularly especially where clients may have not visited the salon in 6 months or had a change in health or medication. Sensitivity can occur at any point.

If reaction occurs, then remove the tint immediately and wash the area with water. Seek medical attention for your client.

Lash tinting

There are many different colours that can be used individually or blended together for a customised look. Results will depend on 3 things: the natural colour of the client's hair, the colours used to tint, and the amount of time the tint is on the hair. For example, if you are working with a client who has light blonde hair and combine Red and Brown and only process the hair for a minute and one-half, you will achieve a dark blond shade. But if you are working with a client with brown hair and process for a longer period of time, you will achieve a different outcome—a dark brownish red.

Lash tint Procedure

Client should be sitting upright in a chair or on a massage table that is positioned upright.
Cleanse the eyelashes and eyebrows with an eye makeup remover to remove all traces of oils, makeup and dirt.
Ask the client to look up and apply a thin layer of petroleum jelly or eye cream and put the eye protector pads in place underneath the bottom lashes.
Mix the desired colour tint.

Using a tint brush apply the tint to the bottom lashes first on both eyes from base to tip. When you have covered all bottom, lashes ask the client to close their eyes.
Apply tint to the clients lashes on the upper lids from base to tip.

If tint accidently gets into the eye, remove the tint immediately and flush the client's eyes out with saline eye wash.

If your client has booked both an eyelash and brow tint, proceed at this point to application on the brow.

developing time will differ depending on manufacturers instructors. However most commonly tint can be left on for 10-15 minutes.
Remove the excess tint carefully using dampened cotton pads.
Remove the protective pads from under the eye.
With the client's eyes still closed continue to remove the tint with dampened cotton wool until no residue is visible.
Hand dampened cotton wool to your client so that they can also wipe over their eye to remove any tint missed. The client may open their eyes.
Finish with a hydrating eye cream around the eye cream.

Brow tint procedure

Always tint brows before shaping. If you shape before tinting the pores will be open and this can dye the hair follicles.

Cleanse the eyebrows using an eye makeup remover to remove dirt, oil and makeup.
Apply tint to the brow using a tint a brush. Try to avoid getting tint onto the skin.
Gently brush through the brow with a mascara wand to ensure all hairs are covered with tint.
Brow hair tints faster than eyelashes. Remove dye after 1 minute to check the colour. If the brows need to be darker re-apply the tint and leave to develop for a further minute or repeat until desired results are achieved.
Remove tint with dampened cotton wool pads.
Shape brows now.

Waxing the Brows

Wash your hands and put on gloves.
Protect the client's hair by putting on a headband.
Remove the clients make up from the brow area.
Cleanse the skin with pre-wax lotion.

Measure and map the client's brows, working with the client to ensure the shape is to their satisfaction.
When you have decided on the shape, you can outline this in white pencil. Check with the client sat upright that the brows are even and that the client is happy to proceed.
Check the temperature of the wax on the inside of your wrist before applying it to the client.
Talc or oil may be applied to the skin for further comfort.
Working from the inner brow, apply the non-strip wax and leave to set. Make sure you use a new spatula every time to prevent cross-contamination of the wax.
Whilst the first application of wax is setting, you may apply wax beneath the brows on each eye. Work as close to your guidelines as possible.
Check the first application of wax is set before removing, ensuring you support the client's skin and remove the wax with a quick flick of the wrist.
Remove the wax in the same method from under each brow arch. Support the client's skin and remove the wax strip quickly.
Work methodically and apply further wax to the bulb of the bulb or above the brow, depending on the desired shape.
Once all the hair has been removed with the wax, use tweezers to remove any missed hairs, fine hairs or to refine the shape more perfectly.
Get the client to check the brows and make any alterations if required.
Brush the brows and trim any long hairs if necessary.
Brows may be tinted if the client wishes to do so.
At the end of the treatment, apply aftercare lotion/oil and give the client the correct aftercare advice.
Recommend any appropriate homecare products and rebook the client in again for 3-4 weeks' time.
Update the client's consultation form.

Aftercare Advice

It is very important to give your client clear instructions about what to expect and what to do at home to ensure they get the best outcome from their treatments. Importance of aftercare should be emphasised, and clients should be provided with clear written aftercare instructions to take home to prevent adverse reactions and know how to deal with them.

They must also have an appreciation of the importance of regular services and be given the opportunity to purchase homecare.

Waxing Aftercare

Advise the client against touching or rubbing the areas immediately after treatment.
If redness occurs, they can apply a damp compress to soothe the area.
Stray hairs can be removed between brow shaping appointments.
Makeup should be avoided on the area for 24 hours.
Non-perfumed soothing gel can be applied.
Heat treatments should be avoided such as saunas, steam rooms and sunbeds for 24-48 hours.
Explain that the client should return in 3-4 weeks for another treatment.

Tinting Aftercare

Do not touch or rub the eyes, eyelashes, or eyebrows immediately after the treatment.
If redness or irritation occurs, apply a cool damp cotton compress to the eye area. If symptoms persist for more than 24 hours seek medical advice.
The effects of the tint will start to diminish within 2 to 3 weeks as hair grows out and the tint colour fades. Strong sunlight/UV light can make the tint colour fade faster.
Avoid washing hair, eyelids, eyelashes, eyebrows, or swimming for 24 hours, as these activities can cause the tint results to fade.
Avoid exfoliating facial cleansers. Those containing AHA's or other chemical exfoliates may decrease the life of the tint. A gentle facial cleanser or eye makeup remover is recommended.
Do not apply makeup or mascara to lashes or receive any other eye treatments for at least 24 hours after the lash or brow tinting treatment.
Try to use oil-free facial products around the eye area to prolong the tint colour.
You can receive a lash or brow tinting treatment every 3 to 4 weeks

Thank you for choosing The Looking Glass Beauty School!

Thank You

Congratulations on completing your
Lash & Brow Tint & Wax Course.

The Looking Glass Beauty

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